

BRIEF 2 OF 3 — HEALTH

Local Government and the Right to Health

Municipalities act on the determinants of health everywhere; only some can make that action durable

Comparative evidence: Turin, Barcelona, Athens | Audience: city networks & municipal governments | June 2026

IN ONE SENTENCE

All three cities studied act decisively on the social determinants of health — housing-adjacent services, urban planning, outreach to the uninsured — but only where decentralisation grants recognised powers, coordination fora, and access to funding, as in Barcelona and to a lesser extent Turin, does that action consolidate into durable programmes rather than remaining ad hoc and donor-dependent, as in Athens.

1. The Policy Problem

Health competences are formally allocated to regional or national health systems in all three jurisdictions examined, leaving municipalities with a narrower constitutional remit than in education or housing. Yet cities materially shape health outcomes through what this research identifies as an urban Health in All Policies approach: parks and air quality, mobility and road safety, tobacco and alcohol environments, heat-wave planning, and registration points that bring the uninsured into primary care. These interventions are frequently "beyond obligation" — no higher-level rule compels their creation — yet the comparative evidence shows they are pivotal for equity. The question this brief addresses is not whether municipalities should be involved in health, since they already are, but how to make that involvement sustainable rather than precarious.

2. What the Comparative Evidence Shows

City	Institutional anchor	Outcome pattern
Barcelona	Article 43 of the Spanish Constitution, a decentralised national health system, and the Consorci Sanitari de Barcelona — a formal intergovernmental health authority.	Municipal authority complements regional entitlements with preventive, community-based programmes that directly address neighbourhood-level inequalities.
Turin	Article 32 of the Italian Constitution and a regionalised national health system (SSN); health integrated with social policy via neighbourhood hubs.	Effective integration of health and social inclusion measures, but effectiveness remains tied to regional funding and cannot resolve spatial inequalities alone.
Athens	Strong national centralisation and chronic underfunding of the Greek national health system (ESY); no equivalent formal municipal health authority.	Reliance on solidarity networks, NGOs, and international donors; municipal clinics and outreach mitigate exclusion but cannot embed universality, continuity, or quality.

2.1 Civil society as structural, not supplementary

In all three cities, NGOs, solidarity networks, and local associations are structural to reaching groups systematically excluded from mainstream provision — migrants, homeless persons, and people who use drugs. The comparative distinction lies in their institutional role: in Turin and Barcelona, these actors complement an existing public health architecture; in Athens, they frequently substitute for absent or underfunded state provision. The latter pattern reveals both the resilience of local health ecosystems built from civil society resources and the accountability risk

of relying on non-state providers where constitutional or statutory backing is weak — services outsourced without clear standards can leave marginalised groups subject to what amounts to a postcode lottery.

2.2 Sexual and reproductive health as a frontline municipal function

Across the cases, municipalities act as first responders in prevention and information services that often fall outside formal obligation: local clinics and community centres frequently provide sexual health counselling, testing for sexually transmitted infections, and youth-friendly information points. These services both fill gaps in national provision and build trust with vulnerable groups who may otherwise avoid formal health institutions altogether.

3. The Limits of Goodwill

The comparative evidence converges on a single constitutional lesson: municipal commitment to health is necessary but not sufficient. Universality is formally recognised in all three jurisdictions, yet this recognition does not automatically translate into equitable access. Where competences and finance remain centralised, as in Greece, municipal initiatives tend to be ad hoc, project-based, and disproportionately vulnerable to political or economic fluctuation. Critically, even in better-resourced contexts, municipal health initiatives often lack systematic monitoring and evaluation, which undermines accountability and limits the capacity to learn from experience and scale what works.

4. Recommendations for City Networks and Policymakers

1. Formalise municipal participation in regional or national health coordination

- a) Barcelona's Consorci Sanitari model — a standing intergovernmental health authority with municipal representation — offers a transferable template. Cities without an equivalent body should advocate for one, since informal cooperation cannot substitute for institutionalised coordination.

2. Anchor Health in All Policies instruments in multi-year budgets

- b) Air quality, mobility, and heat-wave planning should not depend on annual discretionary allocations. National and regional governments should integrate these "beyond obligation" municipal functions into recurring fiscal frameworks rather than treating them as optional add-ons.

3. Codify civil-society partnerships before they become structural by default

- c) Where NGOs already substitute for state capacity, as in Athens, the priority is not to displace them but to formalise their role through multi-year agreements that define referral pathways, minimum standards, and public accountability — converting precarious substitution into recognised co-production.

4. Build systematic monitoring and evaluation into municipal health initiatives

- d) City networks should support shared indicators and evaluation cycles for outreach and prevention programmes, enabling cities to demonstrate impact to funders and to replicate successful models across different constitutional contexts.

5. Use inter-city networks for benchmarking on social determinants

- e) Because health competences are narrower and more uniformly constrained than housing or education, networks such as Eurocities are particularly well placed to support peer learning on the determinants cities can influence regardless of formal health competence — urban design, mobility, and outreach infrastructure.

5. Closing Note

Health policy illustrates with particular clarity a broader argument running through this research: constitutional architecture sets not only the floor of minimum entitlements but also the ceiling of municipal potential. Where municipalities are empowered with legal authority, coordination fora, and stable resources, they translate the right to health into tangible, context-sensitive guarantees; where they are not, even the most committed local administration is left managing the social consequences of a gap it did not create and cannot close alone.